



OFFICIAL

NMHS PROCEDURE

Releasing Severity Assessment Code (SAC) 1 Reports to Patients, Carers, and Family Members

Scope (Audience)	All staff involved in SAC 1 Clinical Incident Reviews and Open Disclosure
Scope (Area)	All NMHS sites and services
Summary	This procedure outlines the criteria and processes for the release of Clinical Incident Review (CIR) reports of Severity Assessment Code (SAC) 1 incidents to patients, carers, and/or family members, outside the formal process under the Freedom of Information (FOI) legislation.
Risk Rating	High
Policy Set	Clinical Governance
Policy Sponsor	Executive Director Public Health and Clinical Excellence
Policy Contact	Manager Clinical Governance and Performance
Version	2.0
Review Date	10/12/2027

COMPLIANCE WITH THIS DOCUMENT IS MANDATORY

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1. Aim

This procedure outlines the criteria and processes for the release of Clinical Incident Review (CIR) reports of Severity Assessment Code (SAC) 1 incidents to patients, carers, and/or family members, outside the formal process under the Freedom of Information (FOI) legislation.

This procedure supports the [NMHS Clinical Incident Management Policy](#), [NMHS Management of Serious Adverse Events Procedure](#) and [NMHS Open Disclosure Policy](#). It aligns with the requirements outlined in the [Health Services Act 2016 \(WA\)](#) and the [WA Health Information Management Policy Framework](#).

Note: For this procedure, the term ‘patient’ refers to a person who has received or is receiving care provided by the North Metropolitan Health Service (NMHS). This includes admitted patients, patients attending the Emergency Department, clients attending clinics, and patients/clients receiving care in the community. The term ‘consumer’ is used as an umbrella term to include a patient, their carer and/or family members.

2. Background

NMHS supports responsible sharing of the final CIR report with the patient, their carer and/or family members, and recommends that this is done within the context of an open disclosure discussion. Refer to [NMHS Clinical Incident Management Policy](#), [NMHS Management of Serious Adverse Events Procedure](#) and [NMHS Open Disclosure Policy](#) for further information.

Release of CIR reports must comply with requirements outlined in the [WA Health Information Access, Use and Disclosure Policy \(MP 0015/16\)](#). This includes the release of the CIR report to the patient, their carer or family members with the consent of the patient, as permitted under section 220(1) of the [Health Services Act 2016 \(WA\)](#).

3. Definitions

Terms	Definitions
Contentious	Refers to the nature of incidents that are likely to draw negative publicity for the organisation and/or the individuals involved.
Sentinel events	A subset of serious clinical incidents that are wholly preventable and have caused, or could have caused, serious harm or death of a patient. Refer to NMHS Clinical Incident Management Policy and a list of Australian sentinel events .
Serious Adverse Event (SAE)	An extraordinary event that could have or did result in serious psychological and/or physical harm (or death) to a consumer, staff member, the community or the organisation ¹ .
Severity Assessment Code (SAC) 1 clinical incident	A clinical incident that has or could have (near miss), caused serious harm or death; and which is attributed to health care provision (or lack thereof) rather than the patient’s underlying condition or illness.

¹ Conway et al., “Respectful management of serious clinical adverse events.”



4. Procedure

All CIR reports, and associated records, are subject to FOI legislation and can be released to the patient, their carer, and/or family members via local FOI processes (refer to [NMHS Management of Freedom of Information \(FOI\) Application Policy](#) for further information).

To ensure appropriate governance when releasing CIR reports outside of the FOI process, SAC 1 incidents are considered to fall into one of two categories with separate governance processes required.

4.1 Categories of SAC 1 Incidents

4.1.1 SAC 1 incident meeting the criteria for a Serious Adverse Event (SAE)

A SAC 1 clinical incident that meets the criteria of an SAE may be immediately obvious by the nature of the event or may initially appear to be a 'standard' event that evolves into an SAE over time due to evolving circumstances.

Additionally, the event itself may be sufficient to reach the threshold of an SAE (e.g. a Sentinel Event, or a catastrophic incident involving multiple patients), or it may be a SAC 1 incident with additional risk factors (such as involvement of a Public Advocate, significant litigation potential, or media interest) that elevates it to SAE level. Please refer to the SAE Procedure for further information.

Reports for SAC 1 incidents meeting the SAE criteria should only be provided to the patient or, if the patient lacks decision-making capacity or is deceased, to the 'person responsible' as defined in the [Guardianship and Administration Act 1990 \(WA\)](#) (refer to [section 4.2.1 SAE SAC 1 incidents](#)).

4.1.2 Non-SAE SAC 1 incident

Non-SAE SAC 1 incidents are all other SAC 1 incidents. For any queries or clarification related to these categories, contact the NMHS Public Health and Clinical Excellence (PHCE) directorate via OfficeofEDPHCE@health.wa.gov.au.

4.2 Processes

The decision to release the CIR report may arise from one of the following situations:

- a request has been made by a patient, their carer, or a family member to be provided with the CIR report of a SAC 1 incident.
- the Open Disclosure Lead or NMHS site/service executive team recommends sharing the CIR report of a SAC 1 incident with the patient, their carer, or a family member.

The decision to release the CIR report should be made by the relevant site/service governance committee (or equivalent) after the report has been finalised (most commonly the Clinical Incident Review Committee, CIRC), and the recommendation to release to the 'Patient/Family' has been confirmed by the SAC 1 panel.

Note: this will be documented in the 'Report Distribution' section of the SAC1 CIR report.

The following sections outline the procedural steps required for releasing a CIR report for an SAE SAC 1 incident and for a non-SAE SAC 1 incident.

4.2.1 SAE SAC 1 incident

Once the relevant local governance committee (or equivalent) has determined that the CIR report should be released and that the circumstances meet the criteria of a SAE SAC 1 incident, the following steps must be followed.

The site/service Safety and Quality Director/Manager/Delegate must:

- (a) **Ensure** the patient has given consent (either expressed or implied) for the release of the CIR report to the requester. In the instances where the patient lacks decision-making capacity or is deceased, the release of the CIR report can only be provided to 'the person responsible' as defined in the [Guardianship and Administration Act 1990](#). This may be different from the person listed as a contact person in the patient's healthcare record and follows the same hierarchy as outlined in the [WA Hierarchy of Treatment Decision-Makers](#).
- (b) **Notify** the [NMHS medico-legal services team](#) of the intention to release the CIR report.
- (c) **Assess** the SAC 1 report for suitability to release i.e. is sensitively written, does not contain blaming language, or excessive jargon. Refer to [NMHS FAQ - SAC 1 Investigations](#) for guidance.
- (d) **Review** the CIR report to ensure that all staff-identifying and patient information (where the patient is not the recipient) is appropriately redacted prior to dissemination or submission. Including but not limited to:
 - Full names
 - Job titles or roles
 - Specific responsibilities or descriptions that may reveal identity
 - Contact details or location-specific references.

Once completed, the CIR report should be labelled as "Redacted" where applicable.

- (e) **Submit** a Contentious Issue Briefing Note (CIBN) via the site/service Executive Director (ED), using the template provided at [Appendix 1](#). The ED PHCE should be consulted for input into the drafting of the CIBN. The SAC 1 report must not be released before the CIBN has been signed by the NMHS Chief Executive (CE).
- (f) The CIBN will also advise the Minister for Health (MfH) of the incident, the intention to release the report, and potential risk to the organisation (see [Appendix 1](#) for template)
- (g) The CIBN must be submitted for approval with the finalised CIR report for noting. The SAC1 report is for noting by the CE only and not for further distribution. The sequence of approvals should be:
 - i. Site/service ED to approve and forward to ED PHCE
 - ii. ED PHCE to approve and return to the site/service ED
 - iii. Site/service ED to send the approved CIBN and SAC1 report to the Office of the CE for final CE approval, copying in [the Office of ED PHCE](#). The email should specify that the SAC1 report is provided for CE information only and is not for further distribution.

- (h) Once approved by the CE, the CIR report of the SAE SAC1 incident will be provided to the patient, their carer, and/or a family member; and the CIBN will be sent to the MfH via usual channels.
- (i) If an electronic copy of the CIR report is to be provided to the requestor, a secure file transfer service (e.g. [MyFX](#)) will be used, wherever possible and practicable, in accordance with the [WA Health Information Security Policy \(MP 0067/17\)](#).

4.2.2 Non-SAE SAC 1 incident

Once the relevant local governance committee (or equivalent) have determined that the CIR report should be released and that the circumstances meet the criteria of a standard SAC 1 incident, the following steps must be followed.

The site/service Safety and Quality Director/Manager/Delegate must:

- (a) **Ensure** the patient has given consent (either expressed or implied) for the release of the CIR report to the requester. In the instances where the patient lacks decision-making capacity or is deceased, the release of the CIR report can only be provided to 'the person responsible' as defined in the [Guardianship and Administration Act 1990](#). This may be different from the person listed as a contact person in the patient's healthcare record, and follows the same hierarchy as the [WA Hierarchy of Treatment Decision-Makers](#).
- (b) **Assess** the SAC 1 report for suitability to release i.e. is sensitively written, does not contain blaming language, or excessive jargon. Refer to [NMHS FAQ - SAC 1 Investigations](#) for guidance.
- (c) **Review** the CIR report to ensure that all staff-identifying and patient information (where the patient is not the recipient) is appropriately redacted prior to dissemination or submission. Including but not limited to:
 - Full names
 - Job titles or roles
 - Specific responsibilities or descriptions that may reveal identity
 - Contact details or location-specific references.

Once completed, the CIR report should be labelled as "Redacted" where applicable.

The site/service Executive Director must:

- (a) **Review** the recommendation to release the report for approval as the delegate of the NMHS CE, as described in the [NMHS Authorisations Delegations and Decision Making Schedule](#). The recommendation to release to the 'Patient/Family' must be confirmed by the SAC 1 panel. (Note: this will be documented in the 'Report Distribution' section of the SAC1 CIR report).
- (b) **Decide** to approve/not approve the release the CIR report to the patient, their carer, and/or family member.
- (c) If an electronic copy of the CIR report is to be provided to the requestor, a secure file transfer service (e.g. [MyFX](#)) will be used, wherever possible and practicable, in accordance with the [WA Health Information Security Policy \(MP 0067/17\)](#).



4.3 Record keeping

All records relating to the release of SAC 1 CIR reports must be managed in accordance with the [NMHS Corporate Records Management Policy](#).

5. Monitoring and Evaluation

Compliance with this procedure is mandatory. There are legislative obligations relating to this procedure where a statutory penalty may apply for non-compliance.

The site/service ED will be responsible for ensuring local compliance with this procedure.

This procedure will be evaluated as required to determine its currency, relevance, effectiveness, efficiency, and sustainability. At a minimum, it will be reviewed **every two years** using a risk management approach in accordance with the [NMHS Policy Governance Framework](#).

6. References

Conway J, Federico F, Stewart K, Campbell MJ. "Respectful Management of Serious Clinical Adverse Events (Second Edition)". IHI Innovation Series white paper. Cambridge, MA: Institute for Healthcare Improvement; 2011. Accessed February 21, 2025.
<https://www.ihl.org/>

7. Related Documents

Government policies	• Guardianship and Administration Act 1990 (WA) • Health Services Act 2016 (WA)
WA Health mandatory policies	• WA Health Information Management Policy Framework • WA Health Information Access, Use and Disclosure Policy (MP 0015/16) • WA Health Information Security Policy (MP 0067/17) • WA Hierarchy of Treatment Decision-Makers
NMHS Policy documents	• NMHS Authorisations and Delegations Schedule • NMHS Clinical Incident Management Policy • NMHS Corporate Records Management Policy • NMHS Management of Freedom of Information (FOI) Application Policy • NMHS Management of Serious Adverse Events Procedure • NMHS Open Disclosure Policy • NMHS Policy Governance Framework
Other	• NMHS FAQ - SAC 1 Investigations • NMHS SAC 1 Clinical Incident Investigation Report • NMHS Serious Incident Notification Template

8. Resources

- [Australian Open Disclosure Framework](#)
- [NMHS Hub MyFX page](#)

9. Document Control

Version	Policy author	Summary of changes	Document Reference	Approval date
1.0		Initial version	D/24/66025	22/03/2024 (Published 27/03/2024)
2.0	R. Tamim & J. Black	Policy reviewed as scheduled 12-month post implementation with amendments: process refined; removed the requirement of a consumer-friendly summary; BN replaced with CIBN; included reference to the NMHS Management of Serious Adverse Events Procedure ; updated definitions to include SAE; links and references updated wherever relevant; content transferred to the new template.	D/26/48229	19/02/2026

This document can be made available in alternative formats on request for a person with a disability.

Within the Department of Health WA, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Appendix 1 Contentious Issue Briefing Note (CIBN)

CONTENTIOUS ISSUE:

Release of Severity Assessment Code 1 Report To Family Requested

you must insert this statement

“THIS IS CONFIDENTIAL PATIENT INFORMATION AND CAN NOT BE RELEASED”

ISSUE: Family/Guardian/Carer request for Severity Assessment Code (SAC) 1 report
(SAC 1 meets criteria for Serious Adverse Event)

KEY MESSAGES:

- In accordance with the *WA Health Clinical Incident Management Policy (2019)*, a robust investigation has been undertaken into the above incident and the report has been completed and endorsed by the delegated authority (see **Attachment 1**).
- The investigation revealed a contribution by healthcare and recommendations were made OR the investigation did not reveal a contribution by healthcare and declassification was requested.
- In alignment with the *Australian Open Disclosure Framework (2014)* and the *National Safety and Quality in Health Service Standards - action 1.12*, Open Disclosure with the patient/family involved is in progress/ is not in progress- (if not reason)
- As part of the process as outlined in the *Australian Open Disclosure Framework (2014)*, “a formal, written report should be provided in a language and communication style that the patient, their family and carers will understand”. It is appropriate to provide the full clinical incident investigation (SAC 1) report as part of the Open Disclosure process in accordance with Section 220 of the *Health Services Act 2016*.
- Clinical incident reports and associated documentation are subject to Freedom of Information (FOI) legislation and may be released to the patient or their next of kin on request regardless of approval by the health service.

CURRENT SITUATION:

- Details of SAC 1 event – date of incident, SAC1 category
- The North Metropolitan Health Service communications team have been briefed about the potential for media involvement. (if applicable) Has media involvement occurred?
- Current patient situation in relation to incident i.e., deceased/inpatient/discharged.
- Open Disclosure progress/latest update
 - *Legal involvement if applicable*
 - *FOI request for medical records if applicable*
- The SAC 1 Report has been reviewed by executive and is suitable for release.



BACKGROUND:

Summary of incident, key issues identified, with summary of recommendations or rationale for declassification request.

CURRENT AS: